

1. Administrative & Study Identification

**Full Study Title:** A Phase IIb, global, randomized, parallel-group, double-blind, placebo-controlled, multicenter study to assess the efficacy and safety of 5 doses of AZD5004 compared with placebo, given once daily as an oral tablet or tablets for a duration of 36 weeks, in adults aged 18 years and above, who are living with obesity (BMI  $\geq$  30 kg/m<sup>2</sup>), or overweight (BMI  $\geq$  27 kg/m<sup>2</sup>) and have at least 1 weight-related comorbidity  
**Short Title/Acronym:** VISTA **Protocol Number:** D7260C00001 **NCT Number:** NCT06579092 **Other Study ID:** 2024-513691-18-00 **Sponsor Name:** AstraZeneca **Collaborator:** Eccogene **Investigational Product:** AZD5004 (elecoglipron) **Phase of Development:** Phase IIb **Medical Monitor:** AstraZeneca Clinical Operations Lead

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2. Study Synopsis & Rationale

**2.1 Study Objective Primary Objective:** To evaluate the percent change in body weight from baseline at 26 weeks and the proportion of participants with weight loss  $\geq$  5% of baseline weight at 26 weeks. **Secondary Objectives:** To assess the overall efficacy, safety, and tolerability of 5 different doses of AZD5004 compared with placebo over a 36-week treatment period.

**2.2 Background & Rationale** To address the growing need for effective, non-injectable treatments for adults living with obesity or overweight with at least one weight-related comorbidity. AZD5004 (elecoglipron) is a novel oral small-molecule GLP-1 receptor agonist. An oral, once-daily formulation with no food-related restrictions aims to support patient adherence and broaden treatment access for weight management and cardiometabolic improvements compared to traditional injectable GLP-1RAs.

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3. Study Design & Methodology

**3.1 Design Framework Study Type:** Interventional **Primary Purpose:** Treatment **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** Randomized, parallel-group

**3.2 Planned Enrollment & Duration Overall Status:** Completed **Target  $\$N\$:$**  Approximately 304 participants **Number of Sites:** Global, multicenter **Study Start Date:** 08-Oct-2024 **Primary Completion Date:** 21-Nov-2025 **Study Completion Date:** 21-Nov-2025

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## 4. Subject Selection (Eligibility Criteria)

**4.1 Inclusion Criteria** 1. Adults  $\geq 18$  years of age, all genders. 2. BMI of (a)  $\geq 30$  kg/m<sup>2</sup>, or (b)  $\geq 27$  kg/m<sup>2</sup> and have a current diagnosis of at least 1 of the following weight-related comorbidities (treated or untreated): (i) Hypertension (ii) Dyslipidemia or hyperlipidemia (iii) CV disease (iv) Obstructive sleep apnea. 3. A stable body weight for 3 months prior to Screening ( $\pm 5\%$  body weight change).

**4.2 Exclusion Criteria** 1. Have obesity induced by other endocrine disorders (e.g., Cushing's syndrome) or monogenic/syndromic obesity (e.g., Prader-Willi syndrome). 2. Received prescription or non-prescription medication for weight loss within the last 3 months prior to Screening, or have a history of/planned bariatric surgery or weight loss device. 3. History of type 1 or type 2 diabetes mellitus, clinically significant inflammatory bowel disease, gastroparesis, severe disease or surgery affecting the upper GI tract, or history of acute or chronic pancreatitis.

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## 5. Intervention & Treatment Plan

**5.1 Investigational Regimen Dosage/Frequency:** Once daily (5 distinct dose levels evaluated) **Route of Administration:** Oral tablet(s) **Duration of Treatment:** 36 weeks

**5.2 Concomitant & Prohibited Therapy Permitted:** Standard stable therapies for existing weight-related comorbidities (e.g., antihypertensives, lipid-lowering agents). **Prohibited:** Other prescription or over-the-counter weight-loss medications and concurrent GLP-1 receptor agonists.

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## 6. Study Timeline & Visit Schedule

| Visit ID     | Window (Days) | Key Activities/Procedures                                                                    |
|--------------|---------------|----------------------------------------------------------------------------------------------|
| Screening    | Day -14 to 0  | Informed Consent, Medical History, BMI measurement, Weight stability verification            |
| Baseline     | Day 0         | Randomization, First Dose, Baseline vital signs and safety labs                              |
| Interim      | Week 26       | Primary Efficacy Assessment, Weight measurement, Safety Labs                                 |
| End of Study | Week 36       | Final Vital Signs, Exit Interview, IP Accountability, Final Safety & Tolerability evaluation |

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## 7. Outcome Measures (Endpoints)

**7.1 Primary Endpoint Definition:** Dual primary endpoints: Percent change in body weight from baseline at 26 weeks AND the proportion of participants with weight loss  $\geq 5\%$  of baseline weight at 26 weeks. **Measurement Method:** Standardized clinical scale assessments at investigational sites.

**7.2 Secondary & Exploratory Endpoints** Change in body weight at 36 weeks. Overall safety, tolerability (particularly gastrointestinal AEs), and pharmacokinetics across the 5 evaluated dosing cohorts.

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## 8. Safety & Adverse Event Reporting

**Adverse Event (AE) Monitoring:** Standard collection via patient diaries and HCP evaluation, closely monitoring for gastrointestinal tolerability typical of the GLP-1RA class.  
**Serious Adverse Events (SAEs):** Standard 24-hour reporting timeline. **Data Monitoring Committee (DMC):** External committee evaluating interim safety signals across the 5 dosing arms.

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## 9. Statistical Analysis Plan (SAP) Summary

**Analysis Populations:** Intent-to-Treat (ITT) for primary weight-loss endpoints; Safety Set for all AE/SAE evaluations. **Sample Size Justification:**  $N = \sim 304$ , powered to detect a statistically significant difference in weight reduction between the AZD5004 dosing arms and placebo. **Handling of Missing Data:** Mixed Models for Repeated Measures (MMRM) and/or Multiple Imputation for subjects discontinuing treatment prior to Week 26.

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## 10. Quality Assurance & Regulatory Compliance

**GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.  
**Monitoring Plan:** Scheduled onsite and remote monitoring visits to ensure protocol adherence and accurate BMI/weight recordings.  
**Audit Trail:** Complete tracking via eCRF locking and query resolution workflows.

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## 11. Study Identifiers & Governance

**Primary ID:** D7260C00001 **Registry Number:** NCT06579092 **Other Study ID:** 2024-513691-18-00 **Sponsor/Lead Organization:** AstraZeneca  
**Collaborator:** Eccogene **Study Title:** VISTA **Official Regulatory Title:** A Phase IIb, global, randomized, parallel-group, double-blind, placebo-controlled, multicenter study to assess the efficacy and safety of 5 doses of AZD5004 compared with placebo, given once daily as an oral tablet or tablets for a duration of 36 weeks, in adults aged 18 years and above, who are living with obesity (BMI  $\geq 30$  kg/m<sup>2</sup>), or overweight (BMI  $\geq 27$  kg/m<sup>2</sup>) and have at least 1 weight-related comorbidity

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12. Clinical Framework (Obesity / Weight Management Specific)

**Primary Condition:** Obesity, Overweight, Diabetes Prevention, Hypertriglyceridemia **Diagnosis Threshold:** BMI  $\geq 30$  kg/m<sup>2</sup>, or BMI  $\geq 27$  kg/m<sup>2</sup> with  $\geq 1$  comorbidity **Medical Methodology:** Pharmacological Therapy (Oral GLP-1RA) **Optimization Target:**  $\geq 5\%$  reduction in body weight at 26 weeks

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13. Study Procedures & Methodology

**Management Pathway:** Standard clinical assessment for weight management and cardiometabolic risk **Education Protocol (HCP):** Administration guidelines for investigational oral GLP-1RA and gastrointestinal AE management **Education Protocol (Patient):** Standard diet and lifestyle counseling recommendations provided alongside the therapeutic intervention **Quality Audit Mechanism:** IP adherence tracking and standardized weighing protocols (e.g., consistent scale, time of day, clothing)

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14. Observation & Follow-Up Schedule

**Total Duration:** 36 treatment weeks **Onsite Visit Cadence:** Baseline, Week 26, Week 36, and scheduled interim safety visits **Remote Monitoring Frequency:** Intermittent check-ins for AE tracking (e.g., Q4W) **Checklist Review Interval:** Regular review of patient compliance, tolerability, and concomitant medications

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15. Sign-off & Approval

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|--------------------------|--------|-----------|-------------|-----|------|-----|-----|--|
| Role                     | Name   | Signature | Date        | :-- | :--  | :-- | :-- |  |
| Principal Investigator   | [Name] | ____      | 03-Apr-2026 |     |      |     |     |  |
| Clinical Operations Lead | [Name] | ____      | 03-Apr-2026 |     | Lead |     |     |  |
| Statistician             | [Name] | ____      | 03-Apr-2026 |     |      |     |     |  |